

cardiology-consultation-note

Nandakumar Cardiology — Consultation Note

Visit Date: 2026-07-10 **Provider:** Priya Nandakumar, MD

History: Patient sustained an anterior STEMI on 2026-04-20, treated with PCI to the LAD. Discharged on guideline-directed medical therapy on 2026-04-25.

Medication Review: Lisinopril 20mg daily and metoprolol succinate 100mg daily, continuous since discharge (2026-04-25) with confirmed adherence and tolerance at follow-up visits on 2026-05-20 and 2026-06-18; no dose reductions required.

Functional Status: NYHA Class II — mild symptoms with ordinary activity, comfortable at rest.

Timing: 82 days have elapsed since the index MI (2026-04-20) to today's visit, exceeding the 40-day minimum waiting period for ischemic etiology. The echocardiogram referenced above (dated 2026-07-01) was performed after this GDMT optimization period, not as an immediate post-MI baseline.

Prognosis: No active malignancy or end-stage comorbidity. Reasonable expectation of meaningful survival beyond 1 year with good functional status.

Additional Review: No revascularization procedure within the past 3 months without a subsequent EF reassessment (PCI was 82 days ago; EF has since been reassessed as above). Not within the 40-day post-MI window.

Assessment/Plan: Ischemic cardiomyopathy with LVEF 28%, NYHA Class II, GDMT-optimized. Recommend ICD implantation for primary prevention of sudden cardiac death.

Electronically signed: Priya Nandakumar, MD, Board-Certified Cardiac Electrophysiologist